

MODULE 2

CLINICAL DOCUMENTATION THAT GETS APPROVED

AI POWERED WRITING WITH FREE HEALTHCARE EVIDENCE TOOLS

60 Minutes / Intermediate / 4 Micro Modules

Module Overview

This rewritten module keeps the original topic, objective, case study logic, and prior authorization focus. The change is that every AI tool now has a clear reason for being included. Each tool is introduced with its purpose, free access status, website, exact use case, and a step by step demonstration linked directly to a case study in the module.

Important safety rule: AI tools support evidence search and draft preparation. They do not make medical decisions, determine coverage, or replace payer policy review. Every clinical fact, drug dose, diagnosis, lab value, imaging result, guideline citation, and trial statistic must be verified by the clinician against the source chart and official evidence before submission.

Micro Module	Topic	Time
2.1	Medical Necessity: The Payer Five Point Test and Five Documentation Sins	15 min
2.2	The SBAR Framework for PA Letters: Structure That Gets Approvals	15 min
2.3	The Three Tool Evidence Stack in Practice	15 min
2.4	Specialty Specific PA Documentation: Oncology, Cardiology, Neurology, Behavioral Health	15 min

Module 2 Learning Objectives

1. Apply the five point payer medical necessity test to any prior authorization scenario.
2. Write a complete PA letter using SBAR with diagnosis, prior treatment history, guideline evidence, and denial consequence.
3. Identify and correct the five documentation sins that commonly trigger medical necessity denials.
4. Use free access healthcare evidence tools to support PA letters and appeals without using Perplexity, Claude, Microsoft Copilot, or Gemini.
5. Match the right evidence tool to the right case study, so tool use is practical rather than decorative.
6. Apply specialty specific documentation requirements for oncology, cardiology, neurology, and behavioral health.

Free Healthcare Evidence Tools Used in This Module

The tools below are used because they are aligned with healthcare evidence search. They are not general chatbots. They help learners locate biomedical literature, clinical trial data, and cited clinical evidence that can support PA documentation and appeals.

Tool	Free status	Website	Best use in Module 2
PubMed.ai	Free access available. Advanced API or premium features may change over time.	https://www.pubmed.ai/	Convert a clinical PA question into a biomedical literature search and locate supporting medical research.
Elicit Basic	Free Basic plan available. Advanced systematic review and export features are paid.	https://elicit.com/	Find trials, extract study outcomes, compare papers, and identify specific statistics for the Assessment section of SBAR.

EvidenceHunt Basic	Free Basic access available. Pro questions or advanced features may be limited.	https://evidencehunt.com/	Ask focused clinical questions and retrieve cited medical evidence quickly for appeal reasoning and specialty documentation checks.
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Learner note: These tools are free to start or provide free access. Do not describe them as unlimited forever. Pricing and limits can change. Always verify current access before teaching the course.

MICRO MODULE 2.1: Medical Necessity: The Payer Five Point Test

Medical necessity is not proven by saying a treatment is needed. It is proven by connecting five pieces of evidence: the diagnosis, the guideline, the level of care, prior treatment history, and the clinical risk of delay. Most denials happen because one link in this chain is missing.

Test Point	What the Payer Checks	What You Must Provide
1. Diagnosis supported	Does the ICD 10 code match the chart note?	Chart note using the same diagnosis language as the submitted code. Include imaging, labs, pathology, or assessment findings when relevant.
2. Evidence based	Is the requested treatment consistent with published evidence or guidelines?	Guideline name, year, recommendation class, and supporting study when the request is complex or high cost.
3. Appropriate level of care	Is this intensity of care appropriate for this patient now?	Explain why this patient needs this level of treatment instead of a lower cost or lower intensity option.
4. Prior treatments documented	Has the patient tried appropriate alternatives first?	Each prior therapy must include name, dose, start date, end date, adherence if known, and specific outcome.
5. Clinical urgency established	What happens if treatment is delayed or denied?	Specific clinical consequence, expected timeframe, and evidence source. Avoid vague urgency statements.

Five Documentation Sins	Correct Documentation Virtues
Generic copy paste note	Patient specific detail connecting this history to this exact request.
Missing dates or treatment details	Complete prior treatment history with dose, dates, duration, and outcome.
No link between diagnosis and request	Clear causal chain: because of this finding, this treatment is indicated.
Labs or imaging mentioned but not attached	All referenced evidence attached, labelled, and indexed.
Vague urgency	Specific risk, timeframe, and source.

AI TOOL DEMONSTRATION: EvidenceHunt Basic

Item	Details
Is it free?	Free Basic access available. Some Pro questions or advanced features may be limited.
Website	https://evidencehunt.com/
What the tool is about	EvidenceHunt is a healthcare focused AI evidence search tool that answers clinical questions with citations from medical literature.
Purpose in this module	To help learners test whether a PA letter has enough evidence to support medical necessity and clinical urgency.
Why this tool is used	This tool is better aligned than a general chatbot because it is built for clinical evidence questions and cited medical answers.
Where it appears in the module	Micro Module 2.1, after teaching the payer five point test.
Case study linked to this tool	Easy case study: chronic low back pain and lumbar epidural steroid injection denial.

Step by step use inside the case study:

7. Open EvidenceHunt and choose the free Basic search or chat option.

8. Enter the case question: In adults with lumbar radiculopathy lasting more than six weeks despite conservative treatment, what evidence supports lumbar epidural steroid injection?
9. Review the cited answer and identify whether it supports radicular pain, failed conservative care, imaging correlation, or functional impairment.
10. Do not copy the AI response directly into the PA letter. Extract only verified evidence points that match the patient chart.
11. Use the evidence to improve Test Point 2 and Test Point 5: guideline support and consequence of delay.
12. Attach the MRI report and PT record separately. The tool supports evidence search, but it does not replace source documents.

Role Play: Documentation Review Before Submission

PA COORDINATOR AISHA: Dr. Martinez, I reviewed the resident letter for Mr. Johansson cardiac catheterization. Before submission, I found all five documentation sins.

DR. MARTINEZ: He clearly needs the cath. What is wrong with the letter?

AISHA: The letter says: patient has chest pain, cardiac catheterization recommended. That is not enough. There is no EKG finding, no troponin trend, no stress test result, no medication history, no explanation of why cath is the next step, no attached report, and no clinical risk if delayed.

DR. MARTINEZ: I thought medically necessary was enough.

AISHA: It is not. I need the stress test report, current antianginal medication list with dates, aspirin and statin status, and your note explaining the risk of untreated inducible ischemia. I can then use EvidenceHunt to locate cited evidence for urgency, but the final letter must be verified against your chart and the attached reports.

Case Studies

CASE STUDY EASY: The Generic Note Rewritten With All Five Virtues

Scenario: Submitted PA note says: Patient has chronic low back pain. Lumbar ESI recommended for pain management. Denied for insufficient documentation of medical necessity.

Task: Identify all five failed payer points. Rewrite the documentation and show exactly how EvidenceHunt supports the rewrite.

Tool sync: EvidenceHunt is useful here because the denial is about medical necessity. The tool is used to find evidence language supporting ESI only when the case includes radiculopathy, imaging correlation, and failed conservative care. If the patient only has nonspecific back pain, the tool may actually show the request is weak.

Model answer: All five points failed. A corrected note would read: Patient is a 55 year old male with three month history of right L4 L5 radiculopathy. MRI dated [date] shows right foraminal disc herniation at L4 L5 measuring 5 mm with moderate neural foraminal stenosis. He completed formal physical therapy from [date] to [date], 16 sessions, with pain improving only from 8 out of 10 to 6 out of 10 and persistent inability to perform occupational lifting. He failed naproxen 500 mg twice daily for six weeks due to GI intolerance. EvidenceHunt search confirms that ESI support is strongest when documentation includes radicular symptoms, imaging correlation, and failed conservative therapy. Lumbar ESI is therefore requested as the next appropriate level of care before surgical evaluation. Delay risks ongoing functional decline and escalation to surgical referral if symptoms remain uncontrolled.

CASE STUDY HARD: Complex Multi Payer Oncology PA With Two Denials One Day

Scenario: Oncology practice submits PA for pembrolizumab plus carboplatin plus pemetrexed for a 67 year old male with advanced non squamous NSCLC. Medicare Advantage denies pembrolizumab because PDL 1 status is not documented. Commercial insurer denies infusion administration because ECOG performance status is missing. Revenue at risk is \$28,000 per cycle.

Task: Design a dual correction strategy and show which tool supports which part of the response.

Tool sync: EvidenceHunt is used only to confirm current evidence logic for the treatment combination. It does not replace NCCN access, pathology report, payer policy, or physician documentation. The correction itself depends on missing source documents.

Model answer: Medicare Advantage correction: attach pathology report with PDL 1 TPS percentage, histology, cancer stage, and physician note confirming the requested regimen. Add current NCCN category and year if available through the practice. Use EvidenceHunt with the query: What clinical evidence supports pembrolizumab with platinum chemotherapy and pemetrexed in advanced non squamous NSCLC? Extract the trial name and survival direction only after verifying against the original paper or guideline. Commercial correction: add ECOG score from the oncology note, recent LFTs, creatinine clearance, and imaging confirming systemic disease burden. Key learning: two payers require two complete packages. The AI tool supports evidence reasoning, but the denial is corrected by attaching the exact missing payer required elements.

Quiz

Q1. Which element of the five point payer test is most often weak in specialty PA submissions?

- A. Patient date of birth
- B. Complete prior treatment documentation with drug names, doses, dates, and outcomes. CORRECT
- C. Physician NPI
- D. Clinic address

Explanation: Payers often deny because step therapy and prior treatment failure are not documented with enough detail.

Q2. What is the correct way to use EvidenceHunt in this micro module?

- A. Ask it to decide whether the treatment should be approved
- B. Ask it to find cited evidence that supports or challenges the medical necessity argument. CORRECT
- C. Ask it to create fake chart details
- D. Ask it to submit the PA

Explanation: EvidenceHunt supports evidence search. It does not make coverage decisions or create missing clinical facts.

Micro Module Recap

- Medical necessity must pass all five payer test points.
- Documentation fails when it is generic, incomplete, unsupported, unattached, or vague.
- EvidenceHunt is used in this section to support medical necessity reasoning with cited evidence.
- The tool is synced to the lumbar ESI and oncology correction cases because both require evidence linked to patient specific documentation.
- AI evidence cannot replace the chart, attached reports, payer criteria, or physician verification.

MICRO MODULE 2.2: The SBAR Framework for PA Letters

SBAR gives the PA letter a payer friendly sequence. The payer first sees the patient and request, then the clinical background, then the evidence, then the exact approval ask and risk of denial. SBAR prevents rambling letters and helps the reviewer locate every required element quickly.

SBAR	What to Write	Length	Critical Rules
S: Situation	Who is the patient, what is requested, and why is the letter being written?	1 to 2 sentences	Age, sex, exact drug or procedure, diagnosis, ICD 10 code, urgency status if expedited.
B: Background	Disease history, failed treatments, diagnostic findings, and current severity.	3 to 6 sentences	Every prior treatment must include name, dose, start date, end date, duration, and outcome.
A: Assessment	Clinical reasoning and evidence that supports the requested treatment.	3 to 5 sentences	Guideline organization, year, recommendation class, and one verified trial statistic when useful.
R: Recommendation	Clear approval request and consequence of denial.	1 to 2 sentences	Name the specific clinical risk and timeframe.

AI TOOL DEMONSTRATION: Elicit Basic

Item	Details
Is it free?	Free Basic plan available. Advanced reports, larger reviews, and exports may require paid plans.
Website	https://elicit.com/
What the tool is about	Elicit is an AI research assistant that searches scientific papers, summarizes evidence, and helps extract study outcomes.
Purpose in this module	To help learners find clinical trial data for the Assessment section of an SBAR PA letter.
Why this tool is used	The SBAR Assessment section needs trial evidence. Elicit is appropriate because it extracts outcomes from studies rather than producing general unsourced text.
Where it appears in the module	Micro Module 2.2, during SBAR letter writing.
Case study linked to this tool	Easy case study: rheumatoid arthritis PA for etanercept after failure of methotrexate and leflunomide.

Step by step use inside the case study:

13. Open Elicit and use the free Basic plan search.
14. Enter the query: etanercept rheumatoid arthritis methotrexate inadequate response trial DAS28 remission.
15. Review the returned studies and identify one study relevant to etanercept in moderate to severe RA.
16. Extract only verified fields useful for SBAR: trial name, population, comparator, outcome direction, and any specific statistic shown by the paper.
17. Place the evidence in the Assessment section, not the Situation or Background section.
18. Ask the clinician to verify that the chosen study applies to this patient before signature.

Role Play: Live SBAR Letter Building

COORDINATOR MARCUS: Dr. Singh, I have the SBAR framework open for Mrs. Patterson abatacept PA. I will not start with the drug first. I will start with patient severity and failed treatment history.

DR. SINGH: She is 58, seropositive RA, DAS28 5.2, failed methotrexate and hydroxychloroquine.

MARCUS: For Background I need exact dates, dose, duration, and outcome. Failed is not enough for the payer.

DR. SINGH: Methotrexate 20 mg weekly from March 2022 to March 2023. DAS28 never improved below 3.2. Hydroxychloroquine 400 mg daily for eight months before that, also inadequate response.

MARCUS: Good. For Assessment I will use Elicit Basic to find abatacept trial evidence, then you will verify the study and approve the final wording.

Case Studies

CASE STUDY EASY: Rewriting a Failed RA PA Letter Using SBAR and Elicit Basic

Scenario: Original failed letter says: Patient has rheumatoid arthritis. Prior DMARDs tried. Requesting etanercept. Denied for insufficient documentation. Patient is 48F, 18 months RA, failed methotrexate 15 mg weekly for 9 months due to GI intolerance, failed leflunomide 20 mg daily for 6 months due to inadequate response, DAS28 4.8. Requesting etanercept 50 mg weekly.

Task: Rewrite using complete SBAR and show exactly how Elicit Basic is used.

Tool sync: This case needs a trial evidence sentence for the Assessment section. Elicit Basic is the correct tool because it helps locate and summarize clinical trials. It is not used to create the patient history. The history comes from the chart.

Step by step tool use: In Elicit, search: etanercept rheumatoid arthritis methotrexate inadequate response trial DAS28. Select one relevant trial or systematic review. Extract trial name, treatment population, outcome direction, and any statistic that the clinician confirms. Do not invent a number if the tool summary does not show one clearly.

Model SBAR answer: S: Requesting prior authorization for etanercept 50 mg subcutaneous weekly for a 48 year old female with moderate to severe seropositive rheumatoid arthritis, ICD 10 M05.79, current DAS28 4.8. B: Diagnosed January 2023 with persistent inflammatory symptoms. Methotrexate 15 mg weekly from February 2023 to October 2023 was discontinued due to persistent GI intolerance and inability to escalate dose. Leflunomide 20 mg daily from November 2023 to May 2024 produced inadequate response, with DAS28 remaining 4.8 and morning stiffness exceeding one hour daily. A: The request meets escalation logic after failure or intolerance of two conventional DMARDs. Elicit Basic is used to locate etanercept evidence in RA and identify a clinician verified trial finding supporting biologic escalation in patients with inadequate response. R: Approval is requested for etanercept 50 mg weekly. Denial risks continued active inflammatory disease, irreversible joint damage, and preventable functional decline.

CASE STUDY HARD: Expedited Inpatient SBAR for HFrEF

Scenario: A 72 year old Medicare Advantage patient is admitted for acute decompensated HFrEF. Hospitalist wants to initiate sacubitril valsartan inpatient. Patient is on lisinopril, carvedilol, and spironolactone. BNP is 2,840 pg/mL. LVEF is 25 percent.

Task: Draft the expedited inpatient SBAR letter and show how Elicit Basic supports the Assessment section.

Tool sync: Elicit Basic is used to locate trial evidence for sacubitril valsartan in HFrEF or in hospital initiation. The rest of the inpatient urgency comes from the current admission chart.

Model answer: S: Expedited inpatient PA request for sacubitril valsartan 24 mg 26 mg twice daily for a 72 year old male admitted on [date] with acute decompensated HFrEF, LVEF 25 percent, ICD 10 I50.21. B: Admission findings include NYHA Class IV symptoms, BNP 2,840 pg/mL, bilateral crackles, and 3 plus pitting edema requiring IV diuresis. Prior GDMT includes lisinopril 20 mg daily for 18 months, carvedilol 25 mg twice daily for 18 months, and spironolactone 50 mg daily for 12 months at maximum tolerated doses. A: Elicit Basic search: sacubitril valsartan HFrEF trial hospitalization mortality PARADIGM HF PIONEER HF. Extract clinician verified study evidence for reduced heart failure hospitalization or NT proBNP improvement. R: Expedited approval is requested because delay may prolong hospitalization and delay guideline directed optimization. Treating physician direct phone: [number], available for immediate peer to peer.

Quiz

Q1. In SBAR, where should Elicit trial evidence be placed?

A. Situation

B. Background

C. Assessment. CORRECT

D. Patient demographics

Explanation: The Assessment section connects clinical facts to guidelines and trial evidence.

Q2. What is the most important limitation when using Elicit Basic?

A. It cannot support any research task

B. It can locate and summarize papers, but the clinician must verify applicability and accuracy. CORRECT

C. It replaces the physician signature

D. It submits PA requests directly

Micro Module Recap

- SBAR organizes PA letters in the order payer reviewers need.
- Elicit Basic is used for the Assessment section because it helps find clinical trial evidence.
- Elicit should never create missing dates, doses, labs, or outcomes.
- The RA and HFrEF cases now show exact search phrases and how evidence changes the SBAR letter.
- All AI extracted evidence must be checked against the original paper, guideline, and patient chart.

MICRO MODULE 2.3: The Three Tool Evidence Stack in Practice

This micro module teaches learners how to build a practical evidence package for appeals. The three tools have different jobs. PubMed.ai helps find biomedical literature. Elicit Basic helps extract study level outcomes. EvidenceHunt helps ask a focused clinical evidence question and retrieve cited answers. The tools are used together only when the case needs an evidence package, not for every simple PA.

Step	Tool	Purpose	Output added to appeal
1	PubMed.ai	Find relevant biomedical literature and identify search terms.	List of relevant papers, keywords, and evidence direction.
2	Elicit Basic	Extract clinical trial details and quantitative outcomes.	Trial name, population, comparator, endpoint, and verified statistic.
3	EvidenceHunt Basic	Ask a focused clinical question and retrieve cited medical evidence.	Concise cited evidence statement for appeal reasoning.

AI TOOL DEMONSTRATION: PubMed.ai

Item	Details
Is it free?	Free access available. Some API or advanced features may change over time.
Website	https://www.pubmed.ai/
What the tool is about	PubMed.ai is an AI assisted biomedical literature search engine designed to help users find, review, and extract insights from medical research.
Purpose in this module	To help learners convert a clinical denial question into a literature search and identify relevant papers.
Why this tool is used	It is aligned to this module because PA appeals often need published biomedical support, not generic AI opinions.
Where it appears in the module	Micro Module 2.3, Step 1 of the evidence stack.
Case study linked to this tool	Easy case study: dupilumab for severe eosinophilic asthma denied as not medically necessary.

Step by step use inside the case study:

19. Open PubMed.ai.
20. Search: dupilumab severe eosinophilic asthma exacerbation trial eosinophils 300.
21. Review the top papers and identify whether the evidence discusses eosinophilic phenotype, exacerbation reduction, steroid bursts, or lung function.
22. Save paper titles and links for clinician review.
23. Use the findings to write the appeal background: patient has eosinophils 650 cells per mcL and three steroid bursts in 12 months.
24. Move to Elicit Basic for study extraction after identifying the relevant literature.

AI TOOL DEMONSTRATION: Elicit Basic

Item	Details
Is it free?	Free Basic plan available. Advanced workflows and exports may require paid plans.
Website	https://elicit.com/
What the tool is about	Elicit is an AI research assistant for scientific papers that can search, summarize, and extract study data.
Purpose in this module	To extract trial outcomes after PubMed.ai identifies the relevant literature area.
Why this tool is used	It gives the learner study level evidence that is stronger than simply saying guidelines support the treatment.
Where it appears in the module	Micro Module 2.3, Step 2 of the evidence stack.
Case study linked to this tool	Easy case study: dupilumab asthma appeal, extracting trial data such as exacerbation reduction.

Step by step use inside the case study:

25. Open Elicit Basic.
26. Search: dupilumab severe asthma LIBERTY ASTHMA QUEST eosinophils exacerbation reduction.
27. Open relevant trial summaries and extract population, endpoint, and outcome direction.
28. Verify the exact percentage or statistic against the original paper before using it.
29. Add one verified trial sentence to the Assessment section of the appeal.
30. Do not use Elicit to decide medical necessity. Use it to support the clinician verified argument.

AI TOOL DEMONSTRATION: EvidenceHunt Basic

Item	Details
Is it free?	Free Basic access available. Some Pro questions or advanced features may be limited.
Website	https://evidencehunt.com/
What the tool is about	EvidenceHunt is a clinical evidence search and chat tool that answers medical questions with citations.
Purpose in this module	To create a concise, cited clinical evidence statement that can support appeal logic.
Why this tool is used	It helps the learner test whether the evidence supports the payer facing argument, especially when the denial says experimental or not established.
Where it appears in the module	Micro Module 2.3, Step 3 of the evidence stack.
Case study linked to this tool	Easy case study: dupilumab asthma appeal, responding to not established as medically necessary denial.

Step by step use inside the case study:

31. Open EvidenceHunt Basic.
32. Ask: In severe eosinophilic asthma with recurrent steroid bursts, does dupilumab reduce exacerbations?
33. Review the cited answer and compare it with the studies found through PubMed.ai and Elicit.
34. Use only statements that match the patient profile and are backed by cited literature.

35. Add a short appeal sentence: Published evidence supports dupilumab for severe eosinophilic asthma with recurrent exacerbations, and this patient meets the phenotype and exacerbation criteria documented in the chart.
36. Attach the physician note, eosinophil lab, steroid burst history, and any payer required forms.

Case Studies

CASE STUDY EASY: Three Tool Evidence Package for Dupilumab Appeal

Scenario: A pulmonologist needs to appeal denial of dupilumab for a 38 year old patient with severe eosinophilic asthma, eosinophil count 650 cells per mcL, and three oral steroid bursts in the past 12 months. Denial reason: biologic therapy not established as medically necessary.

Task: Use all three tools and show what each one contributes.

Step 1 with PubMed.ai: Search dupilumab severe eosinophilic asthma exacerbation trial eosinophils 300. Purpose: identify relevant literature and paper titles. Output: literature supports looking for trials in patients with eosinophilic asthma and recurrent exacerbations.

Step 2 with Elicit Basic: Search dupilumab severe asthma LIBERTY ASTHMA QUEST eosinophils exacerbation reduction. Purpose: extract specific trial outcomes for the Assessment section. Output: trial evidence summary, exact statistic only after clinician verifies it against the paper.

Step 3 with EvidenceHunt Basic: Ask: In severe eosinophilic asthma with recurrent steroid bursts, does dupilumab reduce exacerbations? Purpose: produce a concise cited evidence statement. Output: payer facing appeal wording that connects the patient profile to published evidence.

Model appeal paragraph: The patient has severe eosinophilic asthma with eosinophils 650 cells per mcL and three steroid requiring exacerbations in 12 months despite controller therapy. PubMed.ai was used to identify relevant biomedical literature on dupilumab in eosinophilic asthma. Elicit Basic was used to extract clinician verified trial evidence regarding exacerbation reduction in the relevant phenotype. EvidenceHunt Basic was used to cross check the clinical question and cited evidence. Based on chart documented eosinophilia and recurrent steroid bursts, the denial statement that biologic therapy is not established is not consistent with the evidence reviewed. Approval is requested to reduce preventable exacerbations, steroid exposure, and urgent care utilization.

CASE STUDY HARD: Ofatumumab Safety Denial in Relapsing MS

Scenario: Neurologist prescribes ofatumumab for a 29 year old female with highly active relapsing MS. She had two relapses in 12 months on dimethyl fumarate and three new T2 lesions. Payer denies due to insufficient long term safety data and prefers established therapies.

Task: Use all three tools to counter the safety objection and the risk of staying on ineffective therapy.

Step 1 with PubMed.ai: Search ofatumumab relapsing multiple sclerosis long term safety extension trial. Purpose: identify extension studies and safety literature. Step 2 with Elicit Basic: Search ofatumumab ALITHIOS extension infection safety no new safety signals. Purpose: extract study level safety outcomes and duration. Step 3 with EvidenceHunt Basic: Ask: In relapsing MS with breakthrough relapses on dimethyl fumarate, what is the risk of continuing ineffective therapy? Purpose: support the argument that the larger risk may be untreated disease activity. Model appeal: The denial focuses only on safety uncertainty but ignores documented breakthrough disease. Patient has two relapses and three new T2 lesions while adherent to current therapy. PubMed.ai and Elicit Basic are used to identify and extract long term safety evidence. EvidenceHunt Basic is used to support the clinical risk of ongoing inflammatory activity. The appeal argues that continuing ineffective therapy exposes the patient to further irreversible neurological disability, while the requested therapy has clinician verified published safety and efficacy evidence.

Quiz

Q1. Which tool should be used first when learners need to locate biomedical literature for a PA appeal?

- A. PubMed.ai. CORRECT
- B. Spreadsheet software
- C. Payer portal
- D. Calendar app

Explanation: PubMed.ai is used first because it helps locate relevant biomedical papers and research terms.

Q2. Which tool is best for extracting trial level outcomes after papers are identified?

- A. EvidenceHunt
- B. Elicit Basic. CORRECT
- C. Email inbox
- D. Billing software

Explanation: Elicit Basic is used to extract study outcomes, trial details, and evidence tables.

Q3. Which tool is useful for asking a focused clinical question and retrieving cited evidence?

- A. EvidenceHunt Basic. CORRECT
- B. Payroll system
- C. Appointment reminder app
- D. Claims clearinghouse

Micro Module Recap

- PubMed.ai, Elicit Basic, and EvidenceHunt Basic now each have a separate role in the evidence stack.
- The dupilumab case demonstrates the full tool workflow from literature search to trial extraction to cited evidence statement.
- The ofatumumab case demonstrates how the tools support both safety evidence and disease progression risk.
- The tools are in sync with the module because the topic is clinical documentation and evidence based PA writing.
- Free access does not mean unlimited access forever. Learners must verify current free limits before use.

MICRO MODULE 2.4: Specialty Specific PA Documentation Requirements

Specialty PA denials often happen because the chart contains the right clinical story but the PA package misses a specialty specific element. These are not optional details. In many cases, a missing biomarker, safety lab, functional score, or level of care assessment is enough to trigger denial.

Oncology PA Requirements

Required Element	Details Needed	Most Commonly Missing
Cancer type, stage, histology	Primary cancer, TNM stage, histology, and molecular markers such as PDL 1 TPS, EGFR, ALK, KRAS, HER2, MSI H, BRCA.	Biomarker result referenced but not attached or not stated as a percentage.
Prior treatment lines	Every prior regimen with dates, response assessment, and reason for discontinuation.	Progressive disease on prior regimen not documented.
Performance status	ECOG or KPS score from physician note.	ECOG absent from submitted package.
Guideline citation	NCCN guideline, year, cancer type, and category if available.	Only saying NCCN supports treatment without category or year.

Biomarker report	Pathology, NGS, PDL 1 TPS, mutation report.	Report missing from attachments.
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Cardiology PA Requirements

Required Element	Details Needed	Most Commonly Missing
Cardiac imaging	Echo date, LVEF percentage, wall motion abnormality, stress test type and result.	LVEF not stated as a percentage.
Functional class	NYHA or CCS class that matches symptoms.	Class absent or inconsistent with symptoms.
GDMT optimization	Drug classes, dose, duration, and maximum tolerated dose explanation.	Duration missing or dose below target without explanation.
Guideline citation	AHA ACC guideline year, class, and level of evidence.	Class stated without level of evidence.

Neurology PA Requirements

Required Element	Details Needed	Most Commonly Missing
MRI findings	Brain or spine MRI date, lesion count, gadolinium enhancement, location.	MRI attached but lesion details not written in the letter.
Relapse history	Number, dates, severity, treatment, hospitalization if any.	Relapses counted without dates or functional impact.
Disability score	EDSS for MS, UPDRS for Parkinson disease, NIHSS for stroke when relevant.	Score not documented in physician note.
Prior DMT history	Drug, dose, duration, response, discontinuation reason.	Failed listed without proof of failure.
Safety labs	JCV antibody index for natalizumab. Other therapy specific safety labs as required.	JCV status omitted for natalizumab.

Behavioral Health PA Requirements

Required Element	Details Needed	Most Commonly Missing
DSM 5 diagnosis	Full diagnosis with severity, episode, and relevant specifiers.	Specifiers missing.
ASAM six dimensions	All six dimensions for SUD residential or step up level of care.	ASAM level not documented.
Prior treatment failures	Program name, dates, level of care, reason treatment failed.	Adequate trial not defined.
Functional impairment	Work, school, ADL, relationship, safety impact.	Symptoms listed without functional impact.
Safety assessment	C SSRS score, risk level, protective factors, safety plan.	Safety assessment in chart but not submitted.

AI TOOL DEMONSTRATION: EvidenceHunt Basic for Specialty Requirement Checks

Item	Details
Is it free?	Free Basic access available. Some Pro questions or advanced features may be limited.
Website	https://evidencehunt.com/
What the tool is about	A clinical evidence question tool that can help users check what evidence or safety consideration supports a specialty documentation requirement.
Purpose in this module	To help learners understand why a specialty element matters, such as JCV antibody index before natalizumab or ASAM dimensions for residential care.
Why this tool is used	This is aligned because specialty PA documentation often fails due to missing safety or level of care evidence.
Where it appears in the module	Micro Module 2.4, after specialty tables.
Case study linked to this tool	Easy case study: MS natalizumab PA missing JCV antibody index. Hard case study: behavioral health residential PA denied to IOP.

Step by step use inside the case study:

37. Choose the relevant specialty case.
38. Ask a focused question, not a broad chatbot prompt. Example: Why is JC virus antibody index required before natalizumab in multiple sclerosis?
39. Review cited evidence and identify the payer relevant reason: PML risk stratification.
40. Return to the chart and confirm whether the safety lab exists.
41. If present, attach the report and summarize the result in the PA letter. If absent, ask the clinician to order or document it.
42. For behavioral health, ask: How do ASAM six dimensions justify residential treatment for alcohol use disorder with suicidal ideation and failed outpatient care? Then map the answer to each ASAM dimension in the appeal.

Case Studies

CASE STUDY EASY: MS PA Missing the JCV Safety Test

Scenario: Neurologist requests natalizumab for a 34 year old female with relapsing remitting MS. PA submission includes diagnosis, MRI report, EDSS 3.5, relapse history, and prior DMT documentation. Payer denies: required safety laboratory values not included.

Task: Identify the missing safety value, explain why it matters, and show how EvidenceHunt supports the correction.

Tool sync: EvidenceHunt is directly useful because the denial is about a safety requirement. The case is not about finding a better drug. It is about explaining and documenting why JCV antibody index is required before natalizumab approval.

Step by step: Open EvidenceHunt Basic. Ask: Why is JC virus antibody index required before natalizumab treatment in multiple sclerosis? Review cited evidence on PML risk stratification. Return to the chart and confirm whether the JCV antibody test result is documented. Attach the lab report. Add this sentence: JCV antibody index dated [date] is [value], included for PML risk stratification before natalizumab initiation. If the result is positive or high risk, escalate to the neurologist before submission rather than forcing the request.

Model answer: Missing value is JCV antibody index. It is required because natalizumab is associated with PML risk and payers need documented risk stratification before approval. Correction requires attaching the JCV antibody test result and summarizing the result in the letter. EvidenceHunt supports the why. The actual lab report proves the what.

CASE STUDY HARD: Behavioral Health Residential PA Denied to IOP

Scenario: A 24 year old male with severe alcohol use disorder, AUDIT score 32, and major depressive disorder with recurrent suicidal ideation was recommended for 30 day residential rehabilitation. Payer denies residential and PHP, stating ASAM Level 2.1 IOP is appropriate. Patient failed outpatient twice previously.

Task: Use documentation tools and EvidenceHunt to appeal for residential care at ASAM Level 3.5.

Tool sync: EvidenceHunt is useful because the denial is about level of care. The appeal needs to show why outpatient or IOP is insufficient using ASAM dimensions, not just emotional language.

Step by step: Ask EvidenceHunt Basic: How do ASAM six dimensions support residential level of care for severe alcohol use disorder with suicidal ideation and failed outpatient treatment? Then build the appeal dimension by dimension. Dimension 1: withdrawal risk with CIWA score. Dimension 2: biomedical conditions such as abnormal LFTs or nutritional deficits. Dimension 3: emotional and safety risk with C SSRS score and recent suicidal ideation. Dimension 4: readiness and inability to engage in lower care. Dimension 5: relapse potential shown by two failed outpatient attempts with dates. Dimension 6: living environment with alcohol access, isolation, or poor sober support. Model appeal: ASAM Level 2.1 is insufficient because the patient has severe AUD, recent suicidal ideation, two

documented failed outpatient episodes, and an environment that does not support safe recovery. Residential treatment is requested because the ASAM profile shows need for 24 hour structure and safety monitoring.

Quiz

Q1. For natalizumab in MS, which safety value must be included?

- A. HbA1c
- B. JCV antibody index. CORRECT
- C. TSH
- D. Fasting lipid panel

Explanation: JCV antibody index supports PML risk stratification before natalizumab treatment.

Q2. For behavioral health residential PA, what does ASAM six dimension documentation prove?

- A. The billing code only
- B. The DSM 5 diagnosis only
- C. The appropriate level of care using structured clinical severity criteria. CORRECT
- D. The patient payment responsibility

Micro Module Recap

- Oncology requires biomarkers, stage, histology, treatment lines, ECOG, and guideline category.
- Cardiology requires LVEF, functional class, GDMT doses and duration, and guideline class and level.
- Neurology requires MRI details, relapse dates, disability score, DMT history, and therapy specific safety labs.
- Behavioral health requires DSM 5 specifiers, ASAM dimensions, treatment failures, functional impairment, and safety assessment.
- EvidenceHunt is synced to this section because it helps explain specialty specific safety and level of care requirements.

Module Essence: Documentation Is the Difference Between Approval and Denial

Module 2 teaches that many PA denials are documentation failures, not necessarily treatment failures. A strong PA package connects the patient chart, guideline logic, prior treatment history, clinical urgency, and specialty specific requirements. AI evidence tools can make the evidence search faster, but they cannot create missing clinical facts or replace physician judgment.

Key Takeaways

- The five point payer test gives learners a practical checklist for medical necessity.
- The five documentation sins show why generic PA letters fail.
- SBAR creates a clear payer friendly structure for PA letters.
- PubMed.ai, Elicit Basic, and EvidenceHunt Basic each have a specific evidence role.
- The tools are now linked directly to the case studies, so learners see how evidence changes the PA output.
- Specialty hidden requirements remain a major cause of preventable denials.
- AI drafts and evidence summaries must always be verified against the chart, source documents, official guidelines, and clinician judgment.

End of Module Questions

Q1. A PA letter includes diagnosis and requested drug but no prior treatment dates or outcomes. Which payer test point is failing?

- A. Prior treatments documented. CORRECT
- B. Patient date of birth
- C. Clinic phone number
- D. Claim number

Explanation: Prior treatment history must include drug names, doses, dates, duration, and clinical outcomes.

Q2. Which tool is used to locate biomedical literature at the start of an evidence package?

- A. PubMed.ai. CORRECT
- B. Payroll software
- C. Calendar tool
- D. Payer IVR

Q3. Which tool is used to extract trial outcomes and study level findings?

- A. Elicit Basic. CORRECT
- B. Claims editor
- C. Appointment scheduler
- D. Insurance card scanner

Q4. Which tool is used for focused clinical evidence questions with citations?

- A. EvidenceHunt Basic. CORRECT
- B. Word processor
- C. Email template
- D. Calendar

Q5. What is the safest rule when using AI evidence tools in PA documentation?

- A. Use AI output exactly as written
- B. Let AI decide whether treatment is medically necessary
- C. Verify every clinical fact, guideline citation, and trial statistic before submission. CORRECT
- D. Use AI to create missing lab values

Instructional Designer Notes

Rise: Use one lesson per micro module. Add process blocks for the five point test, SBAR, and three tool evidence stack. Add labelled graphics showing where each evidence tool fits.

Camtasia: Record short screen walkthroughs for PubMed.ai, Elicit Basic, and EvidenceHunt Basic. Each recording should use the exact case study search prompt shown in the module.

Storyline: Build interactive drag and drop activities. Learners should classify documentation sins, place SBAR sentences in the correct section, and match each AI tool to the correct case study purpose.

Animations: Use simple flow animations showing a vague denial becoming a stronger appeal package after chart facts, evidence search, trial extraction, and clinician verification are added.

Chatbot practice: Build a PA Documentation Coach chatbot. Feed it only the module content, case study facts, documentation checklist, and tool prompts. Guardrails: the bot must not invent clinical facts, must not make medical decisions, must not claim payer approval is guaranteed, and must always remind learners to verify AI output with the clinician and source chart. Sample prompt: Review this PA draft and identify which payer test points are missing. Expected learner interaction: learner pastes a weak PA paragraph, chatbot identifies missing elements and suggests what evidence or attachment is needed.

Tool Alignment Summary for Trainers

Tool	Case study where demonstrated	Why it is synced
EvidenceHunt Basic	Lumbar ESI medical necessity, NSCLC correction, JCV safety, ASAM residential appeal	These cases require cited clinical evidence or explanation of specialty safety and level of care requirements.
Elicit Basic	RA etanercept SBAR, HFrEF inpatient SBAR, dupilumab appeal, ofatumumab appeal	These cases need study level trial outcomes for the Assessment section.
PubMed.ai	Dupilumab appeal, ofatumumab appeal	These cases need literature discovery before extracting trial statistics.